

Community shocked after death of local teen



Sean Vetter

By Rosemary MacVicar,
Gazette reporter

The sudden death of Sean Vetter, a popular, fun-loving 17-year-old Almonte high school student, resulting from a freak car accident at the Pakenham railway overpass Sunday night, has shocked and saddened local residents.

The police investigation into the single-car crash into the railway overpass wall on Hwy 15 at Pakenham continues.

Mr. Vetter, who had recently returned from a trip to Jamaica, was driving a 1985 Honda Prelude north on the highway to visit his girlfriend in Arranport when the accident took place at approximately 8:10 pm.

The preliminary Perth OPP

report states that the Honda skidded on the slippery road and struck the overpass wall.

Investigating officer Perth OPP Const George Ball commended Almonte-Ramsay and Pakenham volunteer firefighters' role in extracting Sean from the Honda, which he described as being totally demolished.

The young man was rushed by ambulance to hospital in Ottawa. He died, however, from multiple traumatic injuries on route to the Ottawa Civic Hospital.

The death of this well-known Grade 12 Almonte and District High School student sent shock waves through the community Monday. The announcement of his death came on the first day of classes

following the Christmas break at the high school.

"The school is pretty quiet this morning," said ADHS principal Gerry Tokaruk. "It's sort of a tone that 'I can't believe this has happened,' and the realization that this can happen so quickly."

Mr. Vetter, an avid skier, was a member of the ADHS downhill ski team for three years; Team coach Bob Bird said the teenager belonged to the team's first string. He was considered to be the third or fourth best ADHS skier last year.

"He was exceptionally lively and always enjoying life," said Mr. Bird.

He is survived by his parents Frank and Patricia Vetter of Almonte, his sister Tris (Mrs Greg Wilson) of Ottawa and grandfathers

Louis Vetter of Ottawa and Frank Hennessy of Charlottetown, P.E.I.

Mr. Tokaruk described Mr. Vetter as a teenager with lots of friends.

"He was a very pleasant young man to have in school, one you're proud of having in school," commented the principal.

Const. Ball said police investigation to this date, hasn't revealed how the accident occurred. He noted that Mr. Vetter was familiar with the road. "He certainly knew that overpass and how dangerous it can be," commented the officer.

Police continue to investigate all aspects of the accident, including whether or not a seatbelt was worn. There is no evidence that alcohol or excessive speed were involved, stated Const. Ball.

New doctor comes to town

Dr. Franz Ferraris practised medicine in his native South Africa for six years before moving to Canada in 1982. He says there are professional differences in practice in both countries and his comments reflect his experiences in his homeland. See page 6 for the Gazette's introduction of Almonte's new physician.

Blood donors sought for Jan 15 clinic

Local Red Cross auxiliary officials are hoping the recent controversy and concern over the transmission of the AIDS virus through blood transfusions and donations will not affect the turnout at the blood donor clinic on Wednesday Jan 15th.

The clinic will be held at Naismith school and runs between 2 and 8 pm. High school students are invited to participate between 1 and 2 pm. The clinic is sponsored by the Almonte General Hospital Auxiliary.

Sheila Marsh, the local coordinator for the Red Cross Auxiliary said, "Hopefully the AIDS scare won't have the effect it might have had."

She felt the Red Cross public education programs have been successful in allaying people's fears about the transmission of AIDS in blood transfusion and donation situations.

The Red Cross requests that an annual number of 250 donors respond to the clinics held in Almonte and Carleton Place.

Mrs. Marsh said the last clinic at Naismith a year ago collected 282 units. This is up from the number of donations in 1984 when 225 units were collected.

Mobile clinics are set up daily, Monday to Friday in towns and cities and people in good health between their 17th and 66th birthdays are encouraged to donate blood every three months.

The Red Cross will have a display at the health fair which is being held today (Jan 8th) at Almonte and District High School. They are one of 24 agencies giving displays and hands-on experience to students of area schools.

They will offer a blood testing clinic where students will be able to have their blood tested and typed. Red Cross organizers hope that this display will encourage high school students to donate blood on the 15th.

PUC waits for advice on hydro upgrading

By Joe Banks,
Gazette editor

The Almonte Public Utilities Commission (PUC) is awaiting word from a Port Colborne-based firm on which one of three proposals ranging in cost from \$3.3 to \$1.4 million it should accept in increasing the output of the town's power plant.

The proposals, contained in the Almonte Hydro Development Feasibility Study commissioned by the PUC in the spring of 1984, details ways the town could generate more power through either construction of new generating units or the maintenance and upgrading of the town's current facilities.

If those options are presented to the PUC in January, they will come exactly 60 years to the month when power first came surging from Unit 1 in 1926. The power house was completed in the summer of 1925, and the lifespan of the plant was rated at 40 years, today exceeding the design capacity by 20 years.

The study was completed at a cost of \$31,500, 90 per cent of which was subsidized by the provincial ministry of energy.

PUC foreman Brian Gallagher said the commission has been looking to increase the power plant's output "for years and years" but it wasn't until the government grant became available that the PUC felt the expense could be afforded.

A number of problems are hampering the station's output, Mr. Gallagher said and the firm of Montreal Engineering, which carried out the study, identified these when it carried out the initial inspection in 1982.

Six proposals contained in the study detail solutions to these problems, though, Mr. Gallagher said, the PUC is most serious about three. They include:

- Installation of two new generation units in the existing power house, raising output capacity from about 1,000 kilowatts to 1,800. Cost — \$3.3 million;

- Installation of one new unit in the existing power house, raising the output to 1,480 kilowatts. Cost — \$2 million;

- Maintaining and/or upgrading of the existing units, raising the output to 1,165 kilowatts. Cost — \$1.4 million.

The last appears to be what the PUC

is "leaning toward" Mr. Gallagher said, due to the favorable cost efficiency associated with the proposal.

If accepted, the works would include deepening the forebay above the station at a cost of \$40,000. The commission last summer authorized the digging out of a small part of the forebay at a cost of \$13,000 to determine how "easily" the rock would come out, said Mr. Gallagher.

Upgrading The proposal also recommends upgrading the station intake including building new walls, widening the power canal, installing new trashracks and a new by-pass at a total cost of \$190,000. As for the power house itself, concrete repairs, the construction of a downstream wall and concrete enclosure would cost \$140,000.

Upgrading the first generator unit, which was the original one first installed in 1926, is the most expensive item on the list at \$530,000 after a new runner, shaft, wicket gates, governor, draft tube and rewind generator are replaced.

The second unit's upgrading would cost \$250,000 for the installation of a new runner, governor and draft tube.

The total comes to \$1,150,000, but the estimate for engineering, owner's costs and construction supervision, the report says, would add another hefty \$250,000 to that.

The estimates, said Mr. Gallagher, "are guaranteed" to within 40 per cent of the final costs.

Despite the costs, the PUC agreed something has to be done to remedy the problems handicapping the town's power output. The poor depth of the headpond, the deteriorating concrete at the station's input area, ice jamming up the trashracks in the winter — which requires frequent clearing by PUC crews — and the condition of the units themselves are responsible for the under-exploitation of Almonte's generation resource.

Barber Hydraulics, the Port Colborne firm which originally installed the original generator in the 1920s is, in conjunction with Pitts Construction, doing most of the "legwork" in determining the needs of the generating facilities Mr. Gallagher said.

The company has told the PUC it hopes to be able to advise the commission by Jan 15th of which alternative it intends to recommend.



New Year's baby

Holly Ferrill, 2 and her Mom Cheryl, took a closer look at the newest addition to their family, Brittany Nicole, shortly after her 2:04 am birth Jan 2. Brittany, who weighed in at 8.5 pounds, was the first baby born at Almonte General Hospital in 1986. She was a few

days earlier than parents Cheryl and Peter Ferrill, of Carleton Place, expected, since her arrival was predicted as Jan 5. Brittany is another granddaughter for Elmer and Jean Coyles and Heather and Ted Ferrill, who all live in Carleton Place.

Local doctors echo province-wide opposition to extra-billing ban

By Meryl Eanis,
Gazette reporter

Some Lanark County physicians are saying they feel a sense of betrayal at the provincial government's Health Care Accessibility Act.

Almonte physician Dr. Frank Murphy said "what has been done is disgraceful. Physicians will have no negotiation rights, actually we will become civil servants without civil servants' rights."

"The government wants Cadillac service, but at Volkswagen prices," Dr. Christiane Deschamps, also of Almonte, feels that doctors have a right to extra bill, "especially specialists with extra training and special skills. General practitioners should also have the option to use the OMA fee schedule. Doctors would like to maintain the personal business aspects of their practice."

She pointed out that "... when newspapers print the large amounts that doctors are making, the public

does not realize that in excess of 40 per cent of that amount pays expenses and overhead. It pays the costs of running the business of medicine."

"As the system is designed now, the more patients a physician sees in an hour, the more he/she is paid. To avoid production-line medicine, and give quality care, the doctors want to spend more time with their patients."

"The system does not allow for the extra time sometimes required. I would like to see an alternative method of determining physicians' remuneration," she said.

When asked if he thought strike were a possibility in the event of absolute non-compromise between the OMA and the government, Dr. Alan Drummond, a Perth physician, said that most doctors find the idea of strike action "distasteful."

"Physicians are an honorable group with generally moderate views. We take our work seriously," he said. Dr. Drummond, who is also secretary-

treasurer of the Lanark County Medical Association, added that "strike action is an extremely drastic measure, only an absolute last resort. Hopefully our salvation will be in the courts," he concluded.

The OMA has called a meeting of area representatives for Jan 18th to determine further course of action.

The attitude of the doctors reflects those of many medical professionals all over the province.

On Dec 19th, health minister Murray Elton introduced legislation which will completely ban "extra-billing" for physicians' services, dentistry services performed in hospitals, and all insured optometric services in Ontario.

This Health Care Accessibility Act will also regulate the amounts that physicians can charge for services insured under the Ontario Health Insurance Act.

Anyone who contravenes this is guilty of an offence and liable on conviction to a fine of not more than \$10,000.

According to a statement by Earl Myers, the president of the Ontario Medical Association, its executive committee met on Dec 20th to discuss the implications and ramifications of this legislation.

The OMA is an association of Ontario physicians whose membership is voluntary. One of its purposes is to establish a fee schedule and in the past, it has negotiated the OHIP fee schedule with the government.

Dr. Myers said that "... the so-called Health Care Accessibility Act is not only an encroachment and oppressive use of government power, but also a slur on a profession which has unparalleled record of public service to the people of this province."

Dr. Drummond explained the nature of "extra-billing" and why doctors are against the new legislation.

"Each year, our elected body of representation, the OMA, establishes a fee schedule outlining what a physician

can reasonably charge for various medical acts and services.

"The government in turn, partly through negotiation with our association and based loosely on the OMA fee schedule, establishes its own OHIP fee schedule for those same services."

He said the physician who extra-bills is seeking reasonable compensation for his services, and asks the patient to pay the difference between the OMA schedule of fees and the OHIP schedule.

The crux of the issue is that physicians look at this issue more as a matter of professional freedom and independence. They feel they have the right to deal with their patients directly, without government interference, both medically and financially.

Dr. Drummond said that "within the profession, this right is said to be negotiable and irrevocable. The legislation, by removing this right, for all intents and purposes makes us civil servants (without the benefits of same)

and subject to further controls and restrictions."

"No patient has ever been refused necessary medical care because of an inability to pay. Doctors who extra-bill have always been willing to charge OHIP rates for patients who could not afford the difference," he emphasized.

In terms of the finances in this issue, Dr. Drummond pointed out that the government (implies) that the financial "burden" of those who extra-bill is a staggering threat to the health care budget. In fact, the total cost to the Ontario taxpayer is \$50 million annually, or \$5 per year per citizen in Ontario.

"Taken out of context this sounds like a great deal of money until you compare it to the total provincial health care budget of \$8 billion. Extra billing, therefore, accounts for one half of one per cent of the health care budget."